

Strategic Memo

The Operations Gap in Malaysian Dental Practice

For: Internal planning · Customer conversations **By:** Goodcare Dental Booking Platform **Date:** May 2026

Executive Summary

Malaysian dental clinics already have software for **patient records and billing** — DoctorPartner, Aoikumo, Kreloses, kumoDoc. Those tools cover the clinical side. They do *not* cover the **operational side** of running a clinic, and they certainly do not cover the **business side** of running a franchise.

We are not replacing patient management software. We are building the layer that runs everything else: bookings, staff scheduling, payroll, compliance, marketing, recall, multi-branch reporting. The clinic keeps DoctorPartner. They add us.

This memo identifies the operational pain points that dental owners — especially those running multiple branches — quietly absorb today, ranks them by urgency, and proposes a tiered product roadmap that can be sold immediately on Tier 1 features while signalling the larger vision.

1 · How Owners Currently Work (the spreadsheet stack)

Talk to any dental clinic owner in Malaysia and the back-office workflow looks like this:

- **Bookings** — phone calls, WhatsApp messages, paper diary. *Already covered by our existing product.*
- **Staff schedule** — WhatsApp group, Excel sheet, occasional shouting at lunch.
- **Payroll** — calculator + accountant's spreadsheet, deductions estimated by hand.
- **Doctor commissions** — manual end-of-month tally from the billing software, disputes common.
- **Compliance** — calendar reminders on the owner's phone (if at all). MMC renewal forgotten until the regulator calls.
- **Recall** — non-existent. Patients who don't come back simply churn.
- **Marketing** — Facebook posts when the owner remembers, no idea which ads bring patients.
- **Multi-branch coordination** — Monday morning WhatsApp asking each branch for an Excel summary.

Each of these is a known pain. None has a quality Malaysian-built solution. Most clinics simply absorb the cost as "running a clinic is hard."

2 · The Two Strategic Bets

We are betting on two things being simultaneously true:

1. **Generic global SaaS (Calendly, SimplyBook.me) is too generic** to serve a Malaysian dental clinic well. No IC validation, no PDPA awareness, no MMC tracking, no Bahasa templates.

2. **Local full-suite players (Aoikumo, kumoDoc) are too heavy** — they require migrating an entire practice into their EMR + billing. Most clinics are happy with DoctorPartner; they will not migrate.

We sit in the gap: **modern, branded, Malaysian, deep on operations — but staying out of clinical records and payments.**

That gap is the entire opportunity.

3 · What Franchise Dental Bosses Actually Lose Sleep Over

The following table maps the operational pain points dental owners experience, ranked by **frequency × financial impact × current tooling gap**.

3.1 HR & People management

Pain	Frequency	Cost when missed
Doctor commission disputes	Monthly	Trust erosion, doctor leaves
Payroll deductions (EPF/SOCSSO/EIS) calculated wrong	Monthly	Penalties from KWSP/PERKESO
Leave balance arguments ("I have 5 days left, no you have 3")	Weekly	Morale damage
MMC renewal forgotten	Once	Doctor cannot legally practice
AELB radiology cert expired	Once	RM 50,000 fine + equipment seized
Professional indemnity insurance lapsed	Once	No coverage on next claim
Locum coverage on short notice	Variable	Walk-in patients turned away
CE / CPD hours not tracked	Annual	Doctor registration risk

Conclusion: Doctor commission, leave balance, and compliance reminders are the three HR features that would justify upgrading from a free spreadsheet within the first week.

3.2 Compliance & Regulatory (Malaysia-specific)

Dental clinics in Malaysia must keep track of, at minimum, **nine separate renewal calendars**:

Renewal	Cadence	Consequence of missing
Practice license (KKM)	Annual	Forced closure
Doctor MMC registration	Annual	Doctor cannot practice
MDA membership	Annual	Network access lost
Radiology cert (AELB) per X-ray unit	Annual	RM 50k fine + equipment seized
Professional indemnity insurance	Annual	No coverage on next claim
Fire safety inspection (BOMBA)	Annual	Penalties, possible closure
Sterilisation cycle logs (autoclave)	Daily	Audit findings, KKM warnings

Renewal	Cadence	Consequence of missing
Biohazard waste disposal manifests	Per pickup	RM 10k+ fines
SSM annual return	Annual	Late penalties, struck off
PDPA consent register	Continuous	Up to RM 500k fine per breach

No competitor handles this dental-specifically in Malaysia. A simple "Compliance dashboard" with 60/30/7-day advance reminders would, on its own, justify the entire subscription. One near-miss saved pays for thirty years of platform fees.

3.3 Marketing & Patient Retention

The single largest under-tapped revenue lever in dental: **patients who came once and never returned.**

Capability	Revenue impact
Recall reminders (6-month / 12-month auto-WhatsApp to lapsed patients)	Industry data: clinics with active recall systems see 30–50% more visits per patient lifetime
Win-back campaigns (no visit in > 12 months)	High — recovers churned revenue
Birthday / anniversary auto-greetings	Indirect — patient trust + retention
Lead source tracking (which channel brought this patient?)	Medium — sharpens marketing spend
Loyalty / referral rewards	Medium — drives word-of-mouth
Bulk WhatsApp campaigns with PDPA consent	Medium
Review monitoring (Google, Facebook)	Medium
Patient NPS post-visit	Medium — quality signal

Recall alone funds the platform. A clinic seeing 100 patients/month who would otherwise lose 50% to non-return is missing out on RM 50,000+/year in repeat scaling appointments. RM 150/month subscription is a rounding error against that.

3.4 Financial / Business Intelligence (without touching payments)

Reports derived from already-captured booking and attendance data, with no payment-processing footprint:

- Revenue per doctor / per chair / per branch
- Treatment-mix analysis (margin by procedure)
- Cash-flow forecast for next 30 days based on confirmed bookings
- Outstanding receivables aging (*if owner enters invoice status — not actual payments*)
- No-show financial impact dashboard
- Cost per patient acquisition by channel
- Patient lifetime value
- Chair utilisation rate

DoctorPartner generates some of these for the clinical side. None offer cross-branch consolidation or marketing-ROI attribution.

3.5 Operations

- **Inventory with auto-reorder** — running out of gloves mid-day, ordering anaesthetic the day it expires
- **Lab work tracking** — crown sent to lab, ETA, lab invoice reconciliation against patient charge
- **Equipment maintenance schedule** — autoclave service intervals, compressor checks, X-ray calibration
- **Asset register** — every chair, X-ray, sterilizer with serial, warranty, depreciation
- **Daily checklists** — opening / closing routines, sterile pouch stock
- **Walk-in queue management** — visible to receptionist and doctor
- **Shift handover notes** — morning to evening team continuity

3.6 Multi-Branch / Franchise (the strategic differentiator)

These features only matter once a clinic has two or more branches — but at that point they are *impossible to live without*, and nothing in the Malaysian market handles them well:

- **Cross-branch dashboard** — all branches' revenue, bookings, no-show rate at a glance
- **Patient transfer** between branches with full history continuity
- **Centralised staff roster** — assign locum to whichever branch needs cover
- **Centralised purchasing** — group volume discount on consumables across branches
- **Royalty fee tracking** — franchisor's % of each franchisee's revenue, auto-calculated
- **Brand compliance audits** — logo, signage, uniform, mystery shopper results
- **Per-franchisee onboarding workflow** — new branch open in three weeks not three months
- **Marketing fund pool** — each branch contributes a percentage; spend tracked centrally
- **Inter-branch reporting** — franchisor sees per-branch P&L

3.7 Vendor & Supplier Management

- Vendor directory with contacts, pricing, lead times
- Pricing comparison across suppliers
- Auto-PO generation when stock low
- Lab invoice reconciliation (lab cost vs patient charge — margin per procedure)
- Service contracts (compressor maintenance, autoclave servicing, etc.)

3.8 Customer Experience

- Wait time tracking (booked at 10:00, actually seen at 10:35)
- NPS / patient feedback collection post-visit
- Complaint logs with resolution tracking
- Patient retention rate over time

4 · Proposed Product Roadmap (Tiered Pricing)

Basic — RM 80 / month per clinic

Positioning: *"DoctorPartner sidekick — solve the booking and WhatsApp pain"*

Already built and shipping:

- Online booking, slot management, attendance
- WhatsApp templates + reminders, send tracking
- Mobile home-screen app (PWA)
- Branding & theme customisation
- 1–3 staff accounts

Standard — RM 150 / month per clinic

Positioning: *"Run your clinic, not your spreadsheets"*

Basic, plus the daily operations layer:

- Multiple doctors with individual working hours
- Leave & shift change request workflows
- Duty calendar (week + month views)
- Audit log
- **NEW:** Compliance reminders dashboard ■
- **NEW:** Patient recall (6-month / 12-month auto-flag) ■
- **NEW:** Doctor commission tracking ■

Pro — RM 250 / month per clinic

Positioning: *"Manage like a real business"*

Standard, plus analytics and light HR:

- Revenue by doctor / chair / branch
- Treatment-mix analysis
- No-show financial impact dashboard
- Win-back campaigns
- Lead source tracking
- Patient feedback / NPS
- Locum scheduling
- Certifications expiry alerts
- Light payroll (EPF/SOCSO/EIS calc → payslip PDF generation)

Franchise — RM 400 / month per branch

Positioning: *"Run multiple clinics like one"*

All Pro features per branch + the franchise layer:

- Cross-branch dashboard

- Centralised staff roster
- Patient transfer between branches
- Royalty fee tracking
- Centralised purchasing
- Brand compliance audits
- Per-franchisee onboarding wizard
- Marketing fund pool

Enterprise — Custom

For ten-plus branch operators:

- All Franchise features
- Custom integrations (existing accounting, EMR)
- Dedicated support, SLA-backed
- White-label option

5 · Build Priorities After Current Scope

Ranked by **(value to clinic) × (low build complexity) × (selling power)**:

1. **Compliance reminders dashboard.** Small build, mission-critical feel, RM 50k+ annual savings on one near-miss.
2. **Patient recall reminders.** Directly drives clinic revenue; justifies the subscription on the first returning patient.
3. **Doctor commission tracking.** The single most common dispute between owner and doctor. Bringing transparency builds trust.
4. **Treatment-mix + revenue dashboard.** Once attendance data accumulates, this is data nobody else has.
5. **Light payroll.** EPF/SOCSSO/EIS deductions, payslip PDF — without actual money movement, no banking license needed.

After these five, the product is defensible against both generic SaaS and full-suite EMR competitors.

6 · Why This Is Defensible (the Malaysian Moat)

Several features are essentially **impossible for global SaaS to clone profitably**:

1. EPF / SOCSSO / EIS auto-calculation with current rates and salary bands
2. MMC / MDA / AELB / KKM renewal calendar tracking
3. PDPA consent register with audit trail for marketing communications
4. SST exemption awareness (medical services are zero-rated under MY tax)
5. Bahasa Malaysia and Chinese WhatsApp templates as defaults
6. Bank Negara data residency guidelines (we host in Singapore region — defensible)

7. HRDF levy tracking for clinics with > 10 staff

A global player will never invest in any of this for the size of the Malaysian market. We will. That gap compounds the longer we operate.

7 · Customer Conversation Talking Points

When meeting a dental owner or franchise group, the structure that lands:

1. **Open with the gap.** *"You probably already use DoctorPartner for patient records. That's not what we replace. We handle the parts DoctorPartner doesn't — bookings, your nurses' day, staff scheduling, compliance, recall, multi-branch reporting."*
2. **Probe their pain.** *"How do you currently track when your MMC needs to be renewed? Your AELB cert? Your insurance? Doctor commissions?"* — almost always, the answer is "my own calendar" or "the accountant tells me".
3. **Anchor on revenue impact.** *"Industry data says clinics with recall systems see 30–50% more visits per patient. If you see 100 patients a month and half don't come back, you're leaving roughly RM 50k a year on the table."*
4. **De-risk the decision.**
 - Cancel any month — no contract, no termination fees.
 - 60-day money-back guarantee on the setup fee.
 - Data exports as CSV anytime — never trapped.
 - Doesn't replace DoctorPartner or accounting — runs alongside.
5. **Single ask.** *"Want to try Basic for free for the first month? We'll set up your clinic; you tell me on day 30 if it's helping. Worst case you've lost nothing."*

8 · What to Expect from Customer Conversations

Most owners will ask three things:

Question	Honest answer
<i>"Can you do everything DoctorPartner does?"</i>	No, and we don't want to. We do everything around it.
<i>"What about payroll?"</i>	On the roadmap. We start with the calculation (deductions, commissions); actual disbursement stays in your accountant's hands.
<i>"How long to set up?"</i>	3–5 working days. Onboarding includes training your nurse and importing existing patient list.

The disqualifier we should hear cleanly:

- *"I don't use WhatsApp."* → Different product fit. Move on.
- *"I want one system for everything including EMR."* → Aoikumo / Kreloses fit them better. Move on.

The ideal customer:

- 1–10 doctors, 1–5 branches, already on DoctorPartner or similar

- Owner who spends Mondays consolidating WhatsApp updates from branches
 - Loses sleep over compliance deadlines, leave fights, and doctor commissions
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9 · Open Questions for Founder

To finalise pricing and roadmap, decisions needed:

1. **Confirm tier names + prices.** Basic RM 80 / Standard RM 150 / Pro RM 250 / Franchise RM 400/branch?
 2. **Compliance reminders — Standard or Pro tier?** Recommend Standard (it's the killer feature).
 3. **Recall reminders — same question.** Recommend Standard.
 4. **Doctor commission tracking — Standard or Pro?** Could go either way; Standard makes a stronger story.
 5. **Free trial length.** 30 days, 60 days, or specifically 3 months for first 10 customers?
 6. **Setup fee — fixed RM 1,000, or sliding by tier?**
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10 · Closing

The patient-management software market in Malaysia is crowded. The clinic-operations software market is almost empty. We are not the second-best Aoikumo. We are the first proper operations layer for clinics already using a clinical product they like.

Sell the gap. Build the moat. Charge for the time saved, the compliance averted, and the patients recalled.

— End of memo —